



Rights of mental health consumers

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Functional sub group	Mental health
Summary	The South Metropolitan Health Service is committed to ensuring the rights of mental health consumers are respected by the health service. This commitment includes ensuring consumers are made aware of their rights under the <i>Mental Health Act 2014</i> .
Policy owner	Executive Director Mental Health
Contact	SMHS MH, Manager Safety, Quality, Risk and Policy
Review date	February 2029
NSQHS Standards	Standard 1 - Clinical Governance Standard 2 - Partnering with consumers Standard 5 - Comprehensive Care
Chief Psychiatrists Standards for Clinical Care	Standard 1 - Aboriginal Practice Standard 2 - Assessment Standard 3 - Care planning Standard 4 - Consumer and carer involvement in individual care Standard 5 - Physical health care of mental health consumers Standard 6 - Risk assessment and management Standard 7 - Seclusion and bodily restraint Standard 8 - Transfer of care
Status	Active

1. Background

1.1. About this document

South Metropolitan Health Service (SMHS) mental health services recognise the [Australian Charter of Healthcare Rights](#) applies to all people in all places where healthcare is provided and that being admitted to, or cared for within, a mental health service does not remove these rights. Where the use of the [Mental Health Act 2014](#) (MHA 2014) affects these rights, this policy gives guidance to staff on upholding them in line with the [MHA 2014](#) and the Charter of Mental Health Care Principles.

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1.3. Key definitions

Term	Definition
Charter of mental health care principles	A set of 15 principles scheduled to the MHA 2014 related to person centred care, the recovery approach and the involvement of carers. It is a set of expectations for consumers who receive care and treatment at mental health services. Mental health services must make every effort to comply with the Charter when providing treatment, care or support to persons experiencing mental illness MHA 2014 Part 4, s.11.
Complaint	An expression of dissatisfaction by, or on behalf of, an individual consumer/carer regarding any aspect of a service delivered by a WA health entity, where a response or resolution is explicitly or implicitly expected or legally required.
Confidentiality	Ethical and legal concept that consumer information will only be disclosed to authorised individuals/organisations, at authorised times and in an authorised manner.
Mental Health Advocacy Service (MHAS)	Service established under the MHA 2014 to provide advocacy support to people specified in the Act, including involuntary consumers. It is a free service independent from hospitals and mental health services that provides mental health advocates to help identified mental health consumers to understand their legal rights, express their wishes and uphold their rights.
Mental Health Tribunal (MHT)	An independent decision-making body established under the MHA 2014 . The role of the MHT is to safeguard the rights of people with a mental illness who are involuntary consumers either in hospital or on a community treatment order. The MHT reviews involuntary treatment orders to decide whether a person still needs to be involuntary, approves provision of certain treatments and reviews other decisions affecting a consumer's rights.
Personal possessions	Includes: • articles of clothing, jewellery or footwear • articles for personal use • aids for daily living such as glasses or walking sticks or any medical prostheses which allow the consumer some dignity such as dentures.
Personal support person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult refer to MHA 2014 s.7. Some consumers may use the term kin to refer to a family member.

2. Rights of all mental health consumers

2.1. Provision of information about rights

SMHS mental health services will have processes in place to ensure the consumer is provided with an explanation of their rights:

- as soon as practicable
- communicated appropriately in a language and medium that can be understood by the consumer
- using any means of communication that is practicable
- using an interpreter when necessary
- with appropriate cultural support by engaging with Aboriginal Mental Health Liaison (AMHL)/ Aboriginal Health Liaison Officer (AHLO) to ensure culturally secure communication methods
- accessing available resources for those consumers identifying with a disability
- repeated at the next most appropriate time when the consumer is most likely to receive it.

The explanation of rights will be relevant to the persons individual situation with respect to:

- legal status, detention, examination
- physical examination
- interview with a psychiatrist
- making someone a nominated person
- freedom of lawful communication
- confidentiality
- access to medical record
- access to personal possessions
- making of a complaint
- review of a decision by the Mental Health Tribunal.

Provide the consumer with the relevant written information explaining their rights and document in the consumer's medical record. Refer to the WA Mental Health Commission [brochures](#):

- Being referred to a psychiatrist for an examination
- Community Treatment Orders
- Information for Personal Support Persons
- Information for voluntary patients in hospital
- Inpatient Treatment Orders
- Nominated persons
- Receiving treatment for a mental illness.

The person responsible for providing the explanation must ensure that wherever possible a carer or other PSP of the person is also provided with an explanation of the person's rights, even if the person has not consented to the information being provided. Refer to [MHA 2014](#) s. 245.

SMHS mental health services will ensure a poster of the Charter of Mental Health Care Principles ([refer to Appendix A](#)) is on view within all facilities and that a brochure explaining the Charter is made available to consumers, carers and other PSPs. Refer to [Charter of Mental Health Care Principles brochure](#).

2.2. Physical examination

Mental health consumers are entitled to high quality, evidence-based care and treatment that addresses all aspects of their health including physical health. A mental health consumer has the right to a physical examination on admission to hospital.

Staff must consider cultural protocols including men's and women's business, and where appropriate facilitate connection to Country and cultural healing practices. Consumers may also request treatment from a health professional of the same sex to respect cultural preferences.

Refer to:

- [MHA 2014](#) s.241
- [Chief Psychiatrist Clinical Care Standard: Physical health of mental health consumers](#)
- SMHS Physical health care: mental health consumers guideline (in development)

2.3. Interview with a psychiatrist

At any time, while admitted as an inpatient, a consumer is entitled to an interview with a psychiatrist within a reasonable time of requesting the interview. A psychiatrist may refuse a consumer's request for an interview if the consumer has a history of making repeated requests for interview and the psychiatrist is satisfied that the request is unreasonable. A psychiatrist who grants or refuses to grant an interview must document a record of the interview or the refusal, including any reasons for the refusal. Refer to [MHA 2014](#) s. 260.

2.4. Make someone a nominated person

A mental health consumer has the right to make someone their nominated person under the [MHA 2014](#) if they understand what a nomination is and what it means. This authorises the nominated person to be provided with information, and to be involved in matters relating to the consumers treatment and care to the extent determined by the consumer and nominated person, as long as the psychiatrist believes this is in the consumers best interests. The nomination must be completed on [MHA 2014 Form 12A](#) Nomination of a nominated person. Refer to [MHA 2014](#) Part 16 Division 3.

SMHS mental health will provide the consumer and their nominated person with verbal and written information on the role and rights of the nominated person. Refer to the WA Mental Health Commission brochure:

- [Nominated persons: How to make nominations and what it means.](#)

2.5. Freedom of lawful communication

A mental health consumer has the right to freedom of lawful communication, which includes the freedom to do any of the following in reasonable privacy:

- see and speak with other people in the hospital to the extent that is reasonable
- receive visits from, and otherwise be contacted by, other people at all reasonable times
- have uncensored communications with people, including receiving visits, sending and receiving telephone calls, and sending and receiving mail and electronic communications e.g. email, Facebook
- receive visits from, and otherwise have contact with, their legal practitioner at all reasonable times
- receive visits from, and otherwise have contact with, a mental health advocate at any time.

Refer to [MHA 2014](#) s. 261.

A psychiatrist may only restrict a consumer's freedom of communication if satisfied that the prohibition or limitation is in the consumer's best interests. Refer to [MHA 2014](#) s 262. This excludes the consumers right to have contact with their legal practitioner or a mental health advocate.

A psychiatrist must not restrict a visit by the consumers legal practitioner or mental health advocate unless the psychiatrist is satisfied that there is a serious risk to the safety of the legal practitioner or advocate and there are no other steps which can be taken to reduce that risk. There will be no restriction on other types of communication such as phone call.

An order to prohibit or limit a consumers right of freedom of lawful communication must be documented on a [MHA 2014 Form 12C](#) Restriction on Freedom of Communication and the consumer informed of the order and the reasons for it. The psychiatrist who makes an order to prohibit or limit a consumer's right of freedom of lawful communication must advise the Chief Mental Health Advocate within 24 hours.

An order is current for 24 hours. After this time the form will lapse. The order must be reviewed every 24 hours and a decision made by the psychiatrist to:

- remain in force (for another 24 hours)
- be amended or
- be revoked.

2.6. Confidentiality

SMHS mental health sites will ensure that all information concerning mental health consumers, and management or administrative matters related to consumer care is regarded as confidential by all staff and others involved in care.

Refer to:

- [DoH Information access, Use and Disclosure Policy](#)
- [DoH Patient Confidentiality Factsheet](#)

Wherever possible the SMHS mental health sites will only disclose a consumer's personal information to individuals and organisations with the consumer's consent. Consumer information may be disclosed without consumer consent in circumstances where it is reasonably necessary to:

- prevent serious risk to the life, health or safety of an individual or
- prevent real or immediate risk of danger to the public.

Refer to: [Health Services \(Information\) Regulations 2017](#)

Staff have a duty to maintain appropriate confidentiality of all consumer information they are exposed to in the course of their work. Any breach of confidentiality may lead to disciplinary action being taken, including the matter being treated as serious misconduct.

Refer to:

- [DoH Information Breach Policy](#)
- [DoH Discipline Policy](#)

2.7. Access to medical records

A person provided with treatment or care by a mental health service is entitled to inspect and to be given a copy of any relevant document relating to the person that is in the possession or control of the mental health service. Refer to [MHA 2014](#) s. 243, 248(1). The person in charge of the mental health service must ensure that the request is dealt with as soon as practicable. This right applies in addition to the person's right to apply for information under the [Freedom of Information Act 1992](#). Refer to [MHA 2014](#) s. 248 (2).

For records requested under the *Mental Health Act 2014*, contact your site's Freedom of Information office or Mental Health Release of Information office.

A request to access records may be refused if a psychiatrist reasonably believes any one of the following:

- disclosure would pose a significant risk of serious harm to, or significant risk to the health or safety of, the person or another person
- disclosure would reveal personal information about a somebody else unless the personal information is about an individual who consented to the disclosure of information
- information is of a confidential nature that was obtained in confidence
- the person is a detainee under the [Criminal Law \(Mental Impairment\) Act 2023](#) (CLMI Act 2023) required to be detained at an authorised hospital and the relevant document came into existence under, or for the purposes of, the [Prisons Act 1981](#). Refer to [MHA 2014](#) s. 249.

If a request is refused the reasons for refusal must be written in the consumer's medical record and a copy given to the consumer. The consumer may nominate a medical practitioner or legal practitioner (or both) to inspect and be given a copy of the relevant document. This nominated

person is entitled to inspect and be given a copy of the document as soon as practicable. The nominated person must not disclose any information in the document to the person who has been refused access. Refer to [MHA 2014](#) s. 250, 251.

2.8. Access to personal possessions

Consumers admitted to an authorised hospital must be provided with a secure facility to store personal possessions in and be allowed to access and use those possessions.

The person in charge of the authorised hospital is not obligated to store or to permit use of an item that, in their opinion, may pose a risk of harm to the consumer or another person or that is prohibited or unsafe to store at the authorised hospital.

Refer to: [SMHS Search and Seizure mental health procedure](#)

Property belonging to a consumer which has been left at the hospital for a period of six months following the consumer's discharge may be disposed of by sale or otherwise disposed by the person in charge of the hospital if:

- none of the consumers carer or PSPs claims the article within that six-month period.
- at least one month's notice of the intention to dispose of the article has been given to a consumer and/or carer or PSP

Refer to [MHA 2014](#) s. 259.

2.9. Make a complaint

The ability to make a complaint or question treatment is a fundamental right for all consumers, their carers and other PSPs. SMHS mental health will strive to resolve complaints and identify any aspects of service delivery that require change to effect improvement where possible.

SMHS MH staff will provide consumers with information about providing feedback or making complaints on admission and at all reasonable times afterwards. Engage AMHL / AHLO to provide appropriate support to Aboriginal consumers providing feedback or making a complaint. Written information will be provided on site specific processes for lodging a complaint and the process involved.

Refer to:

- [FSFHG Feedback Form](#)
- [FSFHG Feedback form – Easy Read](#)
- [RkPG Compliments or complaints](#)
- [RkPG Tell us what you think about us – Easy Read](#)
- [PHC Feedback, compliments, concerns and complaints](#)
- [PHC Tell us what you think about us – Easy Read](#)

2.10. Mental Health Tribunal review

SMHS mental health will:

- provide consumers with relevant information regarding the role of the Mental Health Tribunal and how to apply for the Tribunal to review an involuntary treatment order
- support consumers to prepare and attend hearings as required.

Refer to:

- [Your Mental Health Tribunal Hearing](#)
- [My Mental Health Tribunal hearing is over, and I am still an involuntary patient](#)

3. Additional rights for involuntary consumers

3.1. A Treatment Support and Discharge Plan

The treatment, care and support provided to an involuntary inpatient or person under a community treatment order (CTO) must be governed by a care plan, known as a treatment, support and discharge plan as per [MHA 2014](#) s.186.

The Treatment Support and Discharge Plan (TSDP) must outline the treatment and support that will be provided to the consumer while they are subject to the treatment order, and the treatment and support that will be offered once they are no longer on the order. The consumer's psychiatrist must ensure that a TSDP is developed as soon as practicable after the person has been admitted as an involuntary inpatient, or after the CTO has been made, it is reviewed regularly and revised as necessary [MHA 2014](#) s. 187.

The psychiatrist must also ensure that the following people are involved in the preparation of the TSDP:

- the consumer
- parent or guardian if a child
- carer or other PSP
- significant members of the consumer's community (including Elders and traditional healers) and Aboriginal Mental Health Liaison / Aboriginal Health Liaison staff must be included wherever possible if the consumer is Aboriginal, [MHA 2014](#) s. 189
- if the consumer is from a culturally or linguistically diverse population, every effort must be made to identify how best to support the person to participate fully in the development of a culturally informed and appropriate TSDP. This may involve including interpreters or members from the consumer's community to assist with the development of the TSDP.
- where a person has a community care coordinator, they should be involved in the development of the TSDP, as a crucial part of safe handover and continuity of care.
- where a person is being discharged from an inpatient unit on a CTO to a community team, the supervising psychiatrist and treating practitioner who will be continuing the treatment in the community should be involved.
- wherever practicable, any other relevant people suggested by the consumer or central to their life should be involved, for example GPs, support workers, accommodation provider.

Refer to:

- [Chief Psychiatrists Guideline: The preparation, review and revision of treatment support and discharge plans](#)

3.2. A further opinion

All involuntary consumers (inpatients, consumers on a CTO and persons detained under [CLMI Act 2023](#)), their carers or other PSP will be informed of their right to request a further opinion, both verbally and in writing, at the time an involuntary treatment order is issued.

Refer to:

- [Mental Health Commission Further Opinions factsheet.](#)

A further opinion must be independent of the treating team. When a person requests a further opinion, the treating psychiatrist or delegate will discuss options for how a further opinion can be sought and identify and facilitate the preferred option. Options include a further opinion from a:

- psychiatrist within the same health service site.
- psychiatrist from a different site but within the same health service.
- psychiatrist from a different health service.
- private psychiatrist (subject to risk assessment)

The advantages and disadvantages of the options (timeliness, costs and degree of independence) should be explained to the consumer, carer or PSP so that they can make an informed decision. Arrangements for the provision of the further opinion should be made 'as soon as practicable'.

The [Request for a Further Opinion template](#) will be used to document the further opinion to ensure clear communication, consistency and accountability with regard to the process of further psychiatric opinions. The template is to be filed in the medical record and a copy given to the person requesting the further opinion, which may include the consumer, carer or PSP or the treating psychiatrist, and if applicable the MHAS or the Office of the Chief Psychiatrist.

The consumer's psychiatrist will have regard to the recommendations within the further opinion and these recommendations will be explained to the consumer and/or requestor.

If the consumer is dissatisfied:

- a request for reconsideration of treatment can be made to the Chief Psychiatrist
- a request for an additional opinion from the Chief Psychiatrist or their psychiatrist.

The Chief Psychiatrist or psychiatrist can refuse the request for an additional opinion.

Refer to:

- [MHA 2014](#) s.182.
- [Chief Psychiatrists Guideline Further Opinions](#)

3.3. Access to the MHAS

The office of the Chief Mental Health Advocate administers the MHAS and engages other persons to be mental health advocates. Mental health advocates have specific powers to visit mental health services, speak with consumers and other identified persons and make enquiries of mental health service staff.

SMHS mental health staff are required to comply with the [MHA 2014](#) in assisting mental health advocates to exercise their powers and functions under the Act.

The health impact upon Aboriginal people must be considered, and where relevant incorporated and appropriately addressed in the provision of advocacy services

Refer to:

- [SMHS Mental Health Advocates policy](#)

4. Additional rights for voluntary consumers

4.1. Refuse treatment

Voluntary consumers must not be provided with treatment (e.g. medication, counselling, psychological programs) without their informed consent. To give informed consent, the consumer must be able to understand:

- information about the proposed treatment
- the effect of treatment decisions
- and weigh up factors involved in treatment decisions
- communicate treatment decisions.

Where staff consider treatment is required to save the consumer's life or prevent serious physical harm to the consumer or others, treatment may be given without consent.

Refer to:

- [SMHS Emergency Psychiatric Treatment](#)

4.2. Leave hospital

Voluntary consumers may leave hospital whenever they wish with two exceptions. Exceptions include where a voluntary consumer:

- wishes to leave hospital, but a doctor or Authorised Mental Health Practitioner (AMHP) considers the consumer requires an involuntary treatment order they may refer the consumer for an examination by a psychiatrist on a [Form 1A Referral](#). If following review, a psychiatrist determines the consumer does not meet criteria for an inpatient treatment order the consumer will be allowed to leave.
- is in an authorised hospital section 34 of the [MHA 2014](#) allows the person in charge of the ward to make an order to detain a voluntary inpatient in an authorised hospital for assessment on a [Form 2](#). This order allows a voluntary consumer to be detained for up to

six hours for an assessment by a doctor or AMHP to determine if a referral to a psychiatrist is required. The person must reasonably suspect that the voluntary consumer needs an involuntary treatment order.

5. Roles and Responsibilities

The roles and responsibilities of staff have been outlined in this policy. Staff are to ensure they are familiar with what is required for their role.

All staff must complete Aboriginal Patient Centred Care cultural awareness and safety training as per WA Health's Aboriginal Cultural Learning Framework.

6. Compliance monitoring

Each SMHS site Service Director/Co-Director is to ensure compliance with this policy. Compliance monitoring methods may include but is not limited to:

- rate of physical examination within 12 hours of admission to inpatient units
- complaint response times
- audit of TSDPs
- collect, maintain and audit data regarding requests for further opinions
- rate of discharge against medical advice
- monitor engagement of AMHL /AHLO in care planning and discharge processes
- Your Experience of Service feedback.

7. Legislation

A staff member of a mental health service must not ill-treat or wilfully neglect a person who is being provided with treatment or care by the mental health service. Refer to [MHA 2014](#) s. 253.

Penalty: a fine of \$24,000 and imprisonment for two years.

A staff member of a mental health service who reasonably suspects that a reportable incident has occurred in relation to a person who is being provided with treatment or care by the mental health service must report it to:

- the person in charge of the mental health service; or
- the Chief Psychiatrist.

Penalty: a fine of \$6000. Refer to [MHA 2014](#) s. 254.

- [Criminal Law \(Mental Impairment\) Act 2023](#)
- [Carers Recognition Act 2004](#)
- [Mental Health Act 2014](#)
- [Prisons Act 1981](#)

- [Freedom of Information Act 1992](#)
- [Health and Disability Services \(Complaints\) Act 1995](#)
- [Health Services Act 2016](#)
- [Health Services \(Information\) Regulations 2017](#)

8.Related Documents

8.1. WA Health mandatory polices.

- [MP 0124/19 Code of Conduct](#)
- [MP0015/16 Information Access, Use and Disclosure Policy](#)
- [MP 0135/20 Information Breach Policy](#)
- [MP 0127/20 Discipline Policy](#)
- [MP 0130/20 Complaints Management Policy](#)
- [DoH Complaints Management Guideline](#)
- [MP 0051/17 Language Services Policy](#)
- [Language Services Procedure](#)

8.2. SMHS policies

- [Rights of Carers and other personal support persons](#)
- [Mental Health Advocates](#)
- [Emergency Psychiatric Treatment](#)
- [Search and Seizure mental health](#)
- [Discharge against medical advice and patient initiated early discharge](#)

8.3. Other documents

- [Australian Charter of Healthcare Rights](#)
- [Chief Psychiatrists Standards for Clinical Care](#)
- [Chief Psychiatrists Guideline: The preparation, review and revision of treatment support and discharge plans](#)
- [Chief Psychiatrists Guideline Further Opinions](#)

8.4. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP V4	09 February 2026	06 February 2026	2 February 2029	February 2029	Triennial Review

6.6. Approval

Approval by	Executive Director, Mental Health
Approval date	30 January 2026

9. Appendix A: Charter of Mental Health Care Principles



Government of Western Australia
Mental Health Commission



Mental Health Act 2014 Charter of Mental Health Care Principles

Principle 1: Attitude towards people experiencing mental illness	We (a mental health service) must treat people experiencing mental illness with dignity, equality, courtesy and compassion and must not discriminate against or stigmatise them.
Principle 2: Human rights	We must protect and uphold the fundamental human rights of people experiencing mental illness and act in accordance with the national and international standards that apply to mental health services.
Principle 3: Person-centred approach	We must uphold a person-centred focus with a view to obtaining the best possible outcomes for people experiencing mental illness, including by recognising life experiences, needs, preferences, aspirations, values and skills, while delivering goal-oriented treatment, care and support. We must promote positive and encouraging recovery focused attitudes towards mental illness, including that people can and do recover, lead full and productive lives and make meaningful contributions to the community.
Principle 4: Delivery of treatment, care and support	We must be easily accessible and safe and provide people experiencing mental illness with timely treatment, care and support of high quality based on contemporary best practice to promote recovery in the least restrictive manner that is consistent with their needs.
Principle 5: Choice and self determination	We must involve people in decision-making and encourage self-determination, cooperation and choice, including by recognising people's capacity to make their own decisions.
Principle 6: Diversity	We must recognise, and be sensitive and responsive to, diverse individual circumstances, including those relating to gender, sexuality, age, family, disability, lifestyle choices and cultural and spiritual beliefs and practices.
Principle 7: People of Aboriginal or Torres Strait Islander descent	We must provide treatment and care to people of Aboriginal or Torres Strait Islander descent that is appropriate to, and consistent with, their cultural and spiritual beliefs and practices and having regard to the views of their families and, to the extent that it is practicable and appropriate to do so, the views of significant members of their communities, including elders and traditional healers, and Aboriginal or Torres Strait Islander mental health workers.
Principle 8: Co-occurring needs	We must address physical, medical and dental health needs of people experiencing mental illness and other co-occurring health issues, including physical and intellectual disability and alcohol and other drug problems.
Principle 9: Factors influencing mental health and wellbeing	We must recognise the range of circumstances, both positive and negative, that influence mental health and wellbeing, including relationships, accommodation, recreation, education, financial circumstances and employment.
Principle 10: Privacy and confidentiality	We must respect and maintain privacy and confidentiality.
Principle 11: Responsibilities and dependants	We must acknowledge the responsibilities and commitments of people experiencing mental illness, particularly the needs of their children and other dependants.
Principle 12: Provision of information about mental illness and treatment	We must provide, and clearly explain, information about the nature of the mental illness and about treatment (including any risks, side effects and alternatives) to people experiencing mental illness in a way that will help them to understand and to express views or make decisions.
Principle 13: Provision of information about rights	We must provide, and clearly explain, information about legal rights, including those relating to representation, advocacy, complaints procedures, services and access to personal information, in a way that will help people experiencing mental illness to understand, obtain assistance and uphold their rights.
Principle 14: Involvement of other people	We must take a collaborative approach to decision making, including respecting and facilitating the right of people experiencing mental illness to involve their family members, carers and other personal support persons in planning, undertaking, evaluating and improving their treatment, care and support.
Principle 15: Accountability and improvement	We must be accountable, committed to continuous improvement and open to solving problems in partnership with all people involved in the treatment, care and support of people experiencing mental illness, including their family members, carers and other personal and professional support persons.

This document can be made available in alternative formats on request.

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